Pediatrics

Common MCQs

5th Year

MED 06

Growth

&

Development

1. **All the following are associated with short stature EXCEPT:**
    a. Down's syndrome
    b. Rickets
    c. Marfan's syndrome
2. **A 2 years ill child has a head circumference of 40 cm, possible causes included all of the following EXCEPT:**
    a. Rickets
    b. TORCH infection
    c. Familial
    d. Birth asphyxia
    e. Post meningitis
3. **Recognized causes of macrocephaly include all EXCEPT:**
    a. Hydrocephalus
    b. Thalassemia
    c. Down syndrome
    d. Achondroplasia
    e. Meglencephaly
4. **Most common cause of failure to thrive:**
    a. hypoxia due to congenital heart disease
    b. malnutrition
    c. repeated respiratory infections
    d. maternal deprivate
    e. chromosomal abnormalities
5. **12 month old child can EXCEPT:**
    a. Eat biscuits
    b. Can say 10 words
    c. Can hold
    d. Can copy circle
6. **All of the following are abnormal finding in child of 7 months EXCEPT:**
    a. Extensor planter reflex
    b. Moro reflex
    c. Obligatory planter reflex
    d. Head lag
7. **All are true EXCEPT , regarding normal Wt. change in a child:**
    a. Wt. loss in first few days of life
    b. Return to birth wt. by 7-10days
    c. Double birth wt. by 1 year
    d. Quadriple birth wt. by 4 years
8. **A 2 years old child can do the following EXCEPT:**
    a. Help in dressing
    b. Make tower of 2 cubes
    c. Scribbles
    d. Says 3 words sentence
    e. Walk up stairs

9. The following conditions are recognized causes of short stature EXCEPT:

a. Renal tubular acidosis
b. Malabsorption
c. Celiac disease
d. Coartication of the aorta
e. Acute renal failure

10. The average growth of head circumference during the 1st year of life:

a. 4 cm
b. 12 cm
c. 34 cm
d. 25 cm
e. 50 cm

11. By one year of age , a child is able to have all the following foods EXCEPT :

a. Breast milk
b. Peeled fruits
c. Peanuts
d. Eggs

12. Which of the following is most likely to provide the basis for making the correct diagnosis in an infant or child who has failure to thrive (FTT) ?

a. Blood chemistries
b. Cultures
c. History and Physical
d. Radiograph studies
e. None of the above

13. Which of the following statements about precocious puberty is true ?

a. The cause is never found in most cases
b. The children are always shorter than average
c. The incidence is the same for males and females
d. Ketoconazole is the treatment of choice for females
e. All girls with breast development before age 10 require karyotyping for Turner syndrome

14. Which of the following pairs shows the CORRECT stage of normal development ?

a. 6 months – pulls to stand
b. 12 months – pincer grasp
c. 18 months – handedness
d. 2 years – draws a cross
e. 3 years – tells a story

15. All of the following are appropriate in the initial assessment of a child with failure to thrive EXCEPT :

a. Diet history
b. Social history
c. Measurement of height ,weight and head circumference
d. Growth hormone levels
e. Bone age x-ray

16. Which of the following statements about a child with short stature is CORRECT :

a. The bone age is delayed in a child with constitutional growth delay
b. Height crosses a major percentile line on the child's growth chart
c. Weight is more affected than height in a child with an endocrine deficiency
d. Karyotyping is part of the routine investigation of all children with short stature
e. Growth hormone replacement is helpful in a child with familial growth delay

17. If a child can hop and stand on one leg , his age is about :

a. 12 months
b. 18 months
c. 2 years
d. 3 years
e. 4 years

18. Which statement is true about growth :

a. Head growth velocity decrease with advanced age
b. Baby born with 3 kg will be 12 kg after 1 year

19. Macrocephaly :

a. Down syndrome
b. Neurofibromatosis

20. Teething :

a. 6-8 months

21. How much the length of a baby increase in 1 year ?

a. 25 cm

22. One baby the head circumference in 10th percentile and the length also in 10th percentile ?

a. He is normal

23. Which one cause microcephaly ?

a. Trisomy 21
b. Rickets
c. Osteogenesis imperfecta

24. 6 months baby , if cannot do which of the following it's a problem ?

a. Head lag

25. Baby can say "baba" and "mama" and waves "bye bye" at which age ?

a. 10 months

26. 12 months baby, what can he do ?

a. Pincer grasp

27. All the following measurements are true EXCEPT :

a. Head circumference is 35 at birth
b. Weight of a term infant doubles by 4 months
c. The length of the infant is 1 meter by one year

28. All the following can cause short stature EXCEPT :

a. Acute renal failure
b. Chronic renal failure
c. Hypopituitarism

29. The majority of children at age 18 months are expected to do all the following EXCEPT:

a. Sit steadily.
b. Copy a circle.
c. Has a vocabulary of 10 words.
d. Chew a biscuit.
e. Wave bye bye.

30. Recognized cause of macrocephaly include all of the following except :

a. Hydrocephalus
b. Megaencephaly
c. Thalasemia major
d. Achondroplasia
e. Down's syndrome

31. All of the following are causes of macrocephaly EXCEPT :

a. Chronic hemolytic anemia
b. Osteogenesis imperfecta
c. Rickets
d. Down's syndrom
e. Familial

32. All of the following can cause motor developmental delay except :

a. CP
b. Orthopedic problems
c. Emotional neglect
d. Neuromuscular disease
e. Asthma

33. What can an 12 month old baby do :

a. Creep upstairs
b. Cooperates in dressing
c. Identify body part
d. Copy circle
e. Builds a tower of bricks

34. Full term infant double his weight by :

a. 2 months
b. 4 months
c. 7 months
d. 9 months
e. 11 months

35. Child can draw cross by the age of :

a. 18 months
b. 2 years
c. 3 years
d. 4 years
e. 5 years

36. All of the following are true regarding Bottom-shuffling EXCEPT :

a. Often runs in family
b. Tend to walk later than crawling
c. Always follow crawling on all 4
d. Do not need investigations
e. Do not need treatment

37. What is the correct order of copies in children :

a. Circle , square , cross
b. Cross , circle , square
c. Circle , cross , square
d. Square , cross , circle

38. Child ride a tricycle

a. 3 years old.

39. Social smile :

a. 6 weeks.

40. Percent of children-walk when they are 12 month-old unsupported is :

a. 25%
b. 50%
c. 75%
d. 90%
e. 95%

41. What is the most accurate method to measure the child growth :

a. Head circumference
b. Upper to lower segment ratio
c. Bone age
d. High velocity

42. Milestone: at 8th month the baby can:

a. Sit unsupported

43. At 6th month the baby can not:

a. Crawling

44. Delayed bone age, failure to thrive :

a. Hypothyrodism

45. Unilateral breast enlargement in female child :

a. Premature thelarche.

Answers

1)C	**2)E**	**3)C**	**4)B**	**5)D**	**6)?**
7)C	**8)D**	**9)E**	**10)B**	**11)C**	**12)C**
13)?	**14)B**	**15)E**	**16)A**	**17)E**	**18)A**
19)B	**20)A**	**21)A**	**22)A**	**23)A**	**24)A**
25)A	**26)A**	**27)C**	**28)A**	**29)B**	**30)E**
31)D	**32)E**	**33)??B**	**34)B**	**35)D**	**36)??D**
37)C	**38)A**	**39)A**	**40)B**	**41)C**	**42)A**
43)A	**44)A**	**45)A**			

Nutrition

&

Feeding

1. **Kwashiorkor manifestations include all of the following EXCEPT:**
    a. High caloric diet intake
    b. Apathy
    c. Hair changes
    d. Hypotonia
    e. Hypopigmentation
2. **All of the following vitamins are correctly joined EXCEPT:**
    a. Vit.B_{12} ➔ pellagra
    b. Vit.K ➔ high prothrombin
    c. Vit.C ➔ scurvey
    d. Pyridoxine ➔ peripheral neuritis
3. **All are true in kwashiorkor EXCEPT:**
    a. It is defined as body weight < 60% of the excpected
    b. Edema
    c. Hair changes
    d. Marked muscle atrophy
    e. Skin changes
4. **The following statement are true regarding kwashiorkor EXCEPT:**
    a. Wt. for length
    b. It represents adaptation to starvation
    c. Hepatosplenomegaly is due to hepatic fatty infiltration
    d. Skin changes are common
    e. Atrophy of filiform papillae of tongue is usual finding
    f. Asymmetrical tonic neck
5. **Concerning rickets all are true EXCEPT:**
    a. Something about breast feeding for 6 months
    b. 1,25 DiOH vit. D is normal
    c. Ison deficiency is an association
    d. Craniatabe is thinning of the skull
6. **Regarding breast milk all true EXCEPT:**
    a. Protein concentration is higher in mother milk premature baby than mother of mature baby
    b. Foremilk is higher in fat content more than hind milk
    c. Fatty milk contains lactiferrin which inhibits growth of E.coli
    d. Fatty acid pattern of milk reflects maternal diet
    e. Is result in more acidic stool
7. **Nutritional rickets is characterized by all of the following EXCEPT"**
    a. normal or low calcium
    b. raised plasma alkaline phostohatase
    c. raised plasma phosphate
    d. rasied plasma parathyroid hormone level
    e. raised urinary phosphate level

8. **Regarding Kwashiorkor, all the following statements are true EXCEPT:**
    a. Defined as body weight of less than 60% of expected for age
    b. Edema
    c. Sparse fair hair
    d. Marked atrophy of muscle mass
    e. Skin changes are common
9. **All of the following are causes of obesity EXCEPT:**
    a. Chushing syndrome
    b. Hypothyroidism
    c. Corticosteroids treatment
    d. Cystic fibrosis
    e. Familial
10. **All of the following are secreted in breast EXCEPT:**
    a. Lactoferrin
    b. Vit, D
    c. Immunoglobulin A
    d. Vit. K
    e. Little amount of iron
11. **In comparing breast milk and formula, which of the following statements in NOT correct :**
    a. Breast milk has a higher percentage of protein
    b. Breast milk contains leukocytes, complement and lysozymes
    c. Breast milk has a lower concentration of iron than iron fortified formulae
    d. Breast milk has an optimal calcium: phosphorus ration of 2 : 1
12. **Choose the INCORRECT statement about breastfeeding :**
    a. For healthy term babies , breastfeeding is recommended over formula feeding
    b. Breast milk contains more amino acids than cow's milk
    c. Exclusively breastfed babies should be supplemented with iron after 6 months of age
    d. Assessment of adequate intake can be done by counting the number of wet diapers in a day
    e. An HIV-positive mother should not breastfeed
13. **Which of the following problem is most likely to occur in a 12 month-old infant who began taking whole cow milk at 6 months of age :**
    a. Increased frequency of otitis media
    b. Wheezing episodes
    c. Nasal congestion
    d. Decreased serum ferritin level
    e. Diaper dermatitis

14. An absolute contraindication of breast feed is :

a. Acute EBV infection
b. Acute hepatitis A infection
c. Chronic hepatitis B infection
d. Asymptomatic HIV infection
e. None of the above

15. 4 months old baby with rickets , vitamin D deficiency . what is the sign that you are going to see :

a. Bowlegs
b. Small anterior fontanel
c. Craniotabese
d. Dermatitis
e. Microcephaly

16. Breast fed baby of vegetarian mother is susceptible to have deficiency in :

a. Vitamin D
b. Vitamin B
c. Vitamin B12

17. Vegan mother do not take any supplements , the vitamin that will be deficient in her milk is :

a. Vitamin D
b. Vitamin B12
c. Folic acid
d. Vitamin C
e. Vitamin B6

18. Contraindication of breast feeding :

a. Chemotherapy
b. HIV
c. Digoxin
d. Hepatitis B

19. Early sign of rickets :

a. Craniotabes
b. Bow leg

20. One patient has nutritional rickets , he is under breast feeding and he is hypocalcemic and has increased level of alkaline phsphatase enzyme. Which is the correct answer ?

a. Parathyroid hormone

21. Kwashiorkor disease ?

a. Decrease serum albumin

22. Immature milk compared with mature milk ?

a. IgA is more in immature

23. Cow milk is more than human milk in ?

a. Vitamin K

24. Eruption of the teeth is associated with which of the following?

a. Rickets

25. The fluid requirements for 18 Kg child is :

a. 1200
b. 1300
c. 1400
d. 1800

26. All the following are false EXCEPT :

a. Thiamin →Dermatitis
b. Transcopalamine → Cardiomyopathy
c. Vitamin C → Night blindness

27. All are true about kwashiorkor EXCEPT :

a. It is a disease of 2nd child
b. Deficiency of calorie

28. Regarding breast milk , all of the following are true EXCEPT :

a. It contains lactoferrin.
b. Is generally free of microorganisms.
c. Contains IgA.
d. Iron content, although relatively low , is more biologically available.
e. Has a good vitamin K content .

29. All of the following are signs of rickets EXCEPT :

a. Craniotabes.
b. Enlargement of costo-chondral junction.
c. Poor growth .
d. Thickening of wrists and ankles.
e. Conjunctivitis.

30. One of these is a contraindication for the breast feeding ?

a. Chemotherapy
b. Maternal steroidal therapy
c. TB lymphadenitis

31. Clinical feature of rickets include all the following EXCEPT :

a. Obesity

32. Regarding Kwashiorkor, all the following statements are true except :

a. Defined as body weight of less than 60% of expected for age
b. Edema
c. Sparse fair hair
d. Marked atrophy of muscle mass
e. Skin changes are common

33. All is true in Kwashiorkor except:

a. It is defined as a body weight 60% of the expected
b. Edema
c. Hair changes
d. Marked muscle atrophy
e. Skin changes

34. All these diseases can cause FTT except :

a. Renal tubular acidosis
b. Chromosomal disorders
c. Breast feeding
d. Malabsorption syndrom
e. GER

35. In human milk, all is true except :

a. Less proteind than cow's milk
b. Contain IgA
c. Anti-allergic
d. Human's Ca > cow's
e. Lactoferrin is present

36. In kwashiorkor, all the following are true EXCEPT :

a. Usually occurs at age 18-24 months.
b. Caused by energy (calorie) deficiency.
c. Edema.
d. Sparse friable hair.
e. Flaky paint dermatitis.

37. Causes of failure to thrive include all of the following EXCEPT :

a. Renal tubular acidosis.
b. Chromosomal disorder.
c. Breast feeding.
d. Malabsorption.
e. Gastroesophageal reflux.

38. Nutritional rickets is characterized by all of the following EXCEPT :

a. Normal or low calcium
b. Raised plasma alkaline phophatase
c. Raised plasma phosphate
d. Raised plasma parathyroid hormone
e. Raised urinary phosphate level

39. All of the following are causes of obesity EXCEPT:

a. Cushing syndrom
b. Hypothyrodism
c. Corticosteroids treatment
d. Cystic fibrosis
e. Familal

40. All of the following are secreted in breast except :

a. Lactoferrin
b. Vit.D
c. Immunoglobulin
d. Vit.K
e. Little amount of iron

41. Best biochemical indicator for following and assessment of malnutrition is

a. Serum transferring
b. Serum albumin
c. CRP
d. Serum ferritin

42. Cow milk high in ……. more than human milk :

a. Calories
b. Carbohydrates
c. Electrolyte
d. Iron

43. In breast fed newborn, the stool color is:

a. Blue
b. Black
c. Golden yellow
d. Greenish
e. Reddish brown

44. Best indicator of rickets is:

a. ALP level
b. PTH level
c. Calcium level
d. Phosphate level
e. Age

45. Iron absorption is optimum in which?

a. Breast milk
b. Cow's milk
c. Goat milk
d. Yoghurt

46. The most common cause of rickets is :

a. Nutritional
b. Hypophosphatemia
c. Liver disease

47. In kwashiorkor all are true EXCEPT:

a. Weight less than 60%

48. In nutritional rickets the cause is:

a. Hypocalcimia

49. Advantage of breast feeding is:

a. Prevent gastroenteritis

50. Not consistent with rickets :

a. Hyperphosphatemia.

51. Regarding nutritional deficiency:

a. Pellagra – Niacin.

52. Protein intolerance :

a. Never with breast fed.

53. Kwashiorkor all true except :

a. ↑ albumin.

54. Not absolute contraindication for feeding :

a. Bilateral mastitis.

55. 1 year old boy , weight/age is 70% , weight/height is 100% , this child is :

a. Normal
b. Stunted
c. Overweight
d. Underweight
e. Wasted

Answers

1)A	**2)?**	**3)A**	**4)B**	**5)B**	**6)B**
7)C	**8)A**	**9)??D**	**10)D**	**11)A**	**12)C**
13)?	**14)E**	**15)C**	**16)??C**	**17)?**	**18)A**
19)A	**20)A**	**21)A**	**22)A**	**23)A**	**24)A**
25)?	**26)?**	**27)?**	**28)E**	**29)E**	**30)A**
31)A	**32)A**	**33)A**	**34)??C**	**35)All true**	**36)A**
37)C	**38)C**	**39)??D**	**40)D**	**41)?**	**42)??C**
43)C	**44)??A**	**45)A**	**46)A**	**47)A**	**48)A**
49)A	**50)A**	**51)A**	**52)??A**	**53)A**	**54)A**
55)??D					

Vaccination

1. **Which of the following compulsatory vaccine:**
    a. Hemophilus influenza
    b. Pneumococcal vaccine
    c. Meningiococcal vaccine
    d. HAV
2. **The following vaccines are live attenuated, EXCEPT:**
    a. BCG
    b. Pertussis
    c. Measles
    d. Mumps
    e. Oral polio
3. **All are contraindication of vaccination EXCEPT:**
    a. Recent Ig within 2 weeks
    b. Combination immunodeficiency
    c. OPV for hospitalization infants
    d. Pregnant woman
    e. Taking antibiotics
4. **A woman who is positive for hepatitis B surface antigen (HBsAg), but negative for hepatitis B antigen (HBeAg) , delivers at term. What would be best management for this woman's infant ?**
    a. Administer gamma globulin intramuscularly immediately and at 1 month of age
    b. Administer hepatitis B (HB) vaccine immediately and at 1 month and 6 months of age
    c. Administer hepatitis B immunoglobulin (HBIG) if cord blood is positive for HbsAg.
    d. Administer HBIG and HB vaccine immediately , and HB vaccine again at 1 month and 6 months of age
    e. Advise mother that breastfeeding is contraindicated
5. **All of the following groups are contraindications to receive the oral polio vaccine EXCEPT :**
    a. Pregnant women
    b. Asymptomatic HIV-positive children
    c. Household contacts of immunodeficient children
    d. Unimmunized adults at future risk of being exposed to the polio virus
    e. Children with a gammaglobulinemia
    f. The risk of other forms of neurologic illness e.g. Transverse myelitis is extremely small
6. **Vaccine :**
    a. Advised in symptomatic immunocompromised patient
    b. If the patient had anaphylaxis to vaccine , give it again
    c. BCG provide life-long immunity
    d. Varicella is a killed vaccine

7. **Which of the following is NOT correct about the pertussis vaccine :**
    a. Delaying the primary series until the age of 1 year will not reduce the risk of a seizure
    b. The commonly quoted risk of 1:330,000 of brain damage has no basis in fact
    c. It can cause febrile seizure
    d. It may cause infantile spasms or sudden infant death syndrome
8. **Regarding acellular pertussis vaccine , which of the following is true :**
    a. Highly immunogenic
    b. Less expensive
    c. Varicella vaccine can be added
9. **The following vaccines are live attenuated EXCEPT:**
    a. Mumps
    b. BCG
    c. Oral polio
    d. Pertussis
    e. Measles
10. **Contraindication to vaccination include all of the following EXCEPT:**
    a. Patient on antibiotics
    b. Severe combined immunodeficiency
    c. Oral polio vaccination in hospitalized patient
    d. Patient on high dose steroids
    e. Patient with high fever

11. **Which of the following is true about (Hib) vaccine :**
    a. Indicated for high risk children only
    b. Currently available conjugated Hib vaccine that can be given for children as young as 2 months
    c. Children with allergy from egg should not take it
    d. It is recommended that conjugated Hib vaccine should not be administered with history of reaction to DPT vaccine
    e. It is recommended that conjugated Hib vaccine given as single dose without booster
12. **All of the following vaccines are live attenuated EXCEPT:**
    a. BCG
    b. Measles
    c. Mumps
    d. Pertussis
13. **Contraindication of measles vaccination:**

a. Febrile child
b. Diabetic mother
c. Delayed vaccination time
d. Pregnancy

14. Contraindication of Pertussis vaccine :

a. Anaphylactic shock
b. Convulsion

15. Which vaccine is contraindicated in child with egg sensitivity:

a. MMR

16. Pertussis vaccine is absolute contraindicated in:

a. Patient with convulsions during the last 48 h

17. Contraindicated vaccine in immunocompromised child:

a. Oral polio vaccine.

Answers

1)?	**2)B**	**3)E**	**4)??D**	**5)?**	**6)C**
7)?	**8)??B**	**9)D**	**10)A**	**11)B or D**	**12)D**
13)D	**14)A**	**15)A**	**16)A**	**17)A**	

Neonatology

1. **All of the following neonatal reflexes are present at birth EXCEPT:**
    a. Sucking reflex
    b. Moro reflex
    c. Rooting reflex
    d. Placing reflex
    e. Parachuat reflex
2. **Which is not considered normal in new born:**
    a. Erythema toxicum
    b. Mild jaundice on 3rd day
    c. Mangolian spots
    d. Cyanosis on crying
    e. Caput succedaenum
3. **During the newborn period , physical characteristics suggestive of prematurity include each of the following EXCEPT :**
    a. A prominent diastasis of the rectus muscle
    b. Absent breast buds
    c. Thin skin
    d. Hypotonic posture at rest
4. **At 36 hours of age , a full-term infant has not yet passed meconium. Physical examination reveals jaundice and abdominal distension. Radiographs of the abdomen show dilated loops of bowel and calcification in the scrotum. A meconium plug is passed after rectal examination. Which of the following would be the most definitive diagnostic test to order:**
    a. Barium enema
    b. Sweat chloride test
    c. Ultrasound of the abdomen
    d. Urine calcium excretion
    e. Voiding cystourethrogram
5. **Newborn abnormal finding :**
    a. Mangolian spot
    b. Skin dimpling in the sacral region
    c. Erythema toxicum
6. **Normal reflex at birth :**
    a. Asymmetrical moro
    b. Asymmetrical tonic neck
    c. Absent jaw reflex
    d. Font parachute
7. **Lungo hair is normally seen in :**
    a. Normal neonates
8. **Regarding eryhtema toxicarum, which of the following is true ?**
    a. Not usual to be seen before 5 days

b. Can be treated with antibiotics

9. Female newborn has bilateral increase breast size, next step:

a. Prolactine level
b. X-ray
c. Reassurance
d. Squeezing breast
e. Check adrenal tumor

10. Absence of moro reflex in neonatal period:

a. CNS stimulation
b. CNS depression
c. CP
d. Cerebellar ataxia
e. Clavicle fracture

11. Which of the following is not primitive reflex :

a. Moro
b. Grasp
c. Rooting
d. Parachute
e. Tonic neck

12. Reflex appear at 9 month and persist:

a. Moro reflex
b. Parachute reflex

13. Which of the following congenital anomalies usually resolves spontaneously:

a. Club foot
b. Genu varus
c. Tibial torsion
d. Flexible flat feet
e. Genu valgus

14. 2 months old baby in ER with dehydration , which of the following is abnormal skin lesion in a neonate :

a. Skin dumple in the lower sacral region
b. Mongolian spots
c. Transient pustular melanosis
d. Sucking blisters
e. Erythema toxicum

Answers

1)E	**2)D**	**3)D**	**4)B**	**5)B**	**6)C**
7)?	**8)?**	**9)C**	**10)E**	**11)D**	**12)B**
13)C	**14)A**				

Gastroenterology

1. **Signs of moderate dehydration in infant includes all EXCEPT:**
    a. Decrease tearing
    b. Dry mucous membrane
    c. Shock
    d. Depressed anterior fontanel
    e. Oligouria
2. **All the following are recognized cause of constipation in childhood EXCEPT:**
    a. Hirschsprung disease
    b. Hypothyroidism
    c. Spina bifida
    d. Gastroesophageal reflux
    e. Hypercalcemia
3. **The following are cause of chronic diarrhea EXCEPT:**
    a. Cystic fibrosis
    b. Rota virus
    c. Celiac disease
4. **Which of the following support functional recurrent abdominal pain:**
    a. Pain awaking child from sleep
    b. Prolonged fever
    c. Pain radiate to the back
    d. Weight loss
    e. Pain located periumbilical
5. **Regarding gastroenteritis all true EXCEPT:**
    a. Common cause is adenoviruse
    b. Breast feeding should not stop
    c. Isotonic dehydration is common complication
    d. Secondary lactose inrolerance may occure
    e. Treated by oral rehydration solution
6. **Concerning dehydration which statement is not ture:**
    a. Hypernatremia dehydration is associated with doughy skin
    b. Tachycardia is a common sign of dehydration
    c. Serum Na is the major determining factor for blood osmolality
    d. Decreased capillary refill is assign of dehydration
7. **Which of the following is NOT an expected laboratory finding in a child with moderate hyponatremic dehydration:**
    a. serum sodium of 124 meq/L
    b. serum potassium 4.5 meq/L
    c. blood urea nitrogen 30 meq/L
    d. hematocrit is 28%
    e. random blood sugar 5 mmol /L

8. **The true statement regarding acute gastroenteritis in children is:**
    a. Pathogenic E.coli is commonest cause
    b. Drowsiness out of proportion to the degree of dehydration suggests hypertonic dehydration
    c. Marked sunken eyes & diminished skin turgor suggests 5% dehydration
    d. Presence of seizures (convulsion) will exclude the diagnosis
    e. Children should be kept in hospital until the stools have returned to normal
9. **Causes of in childhood include all of the following EXCEPT:**
    a. Hirschsprung's disease
    b. Hypothyroidism
    c. Spina bifida
    d. Hypercalcimia
    e. Gastroesophegeal reflus
10. **A newborn male spits up his first feeding and develops bilious emesis with subsequent feedings. On physical exam he appears ill. Has a scaphoid abdomen and absent bowel sounds. Abdominal X-ray shows air in the distal digestive tract. The most likely cause for this infant's vomiting and clinical findings is :**
    a. Antral web .
    b. Choldochal cyst
    c. Hirschsprung disease
    d. Tracheoesophageal fistula
    e. Volvulus
11. **A 13 month-old infant boy has chronic diarrhea, poor appetite , irritability and growth failure. He had been well previously. Developing normally until diarrhea began at 3 months of age. Findings include weight loss less than 5th percentile and length at 25th percentile, cachectic appearance , wasted extremities and protuberant abdomen. His labs are : albumin 23 , protein 40. Stool positive for reducing sugars and negative for enteric pathogens , ova and parasites. These findings are most consistent with :**
    a. Celiac disease
    b. Cow milk allergy
    c. Crohn's diseases
    d. Cystic fibrosis
    e. None of the above
12. **The most common cause of chronic diarrhea in a 6 – 36 months old child is :**
    a. Toddler's diarrhea
    b. Disaccharidase deficiency

c. Enteric infection
d. Malabsorption e. Protein intolerance

13. Each of the following statements about rotavirus gastroenteritis in children is correct EXCEPT :

a. It usually occurs in the winter months
b. It is self-limited
c. It is often associated with respiratory symptoms
d. It is often accompanied by fever and vomiting
e. It is often associated with grossly bloody diarrhea

14. A 6 week-old male infant develops recurrent episodes of vomiting with dehydration. Each of the following would be consistent with a diagnosis of congenital hypertrophic pyloric stenosis except :

a. Non-bile stained vomitus after feeding
b. A metabolic acidosis and hyperkalemia
c. A history of increasing constipation
d. Good appetite
e. Visible epigastric peristaltic waves

15. Organic causes in the differential diagnosis of chronic abdominal pain includes all the following except :

a. UTI
b. Lactose intolerance
c. Chronic giardiasis
d. Chronic appendicitis
e. IBD

16. A 1 year-old child with a rapid pulse, slightly low blood pressure, cool extremities, oliguria and dry oral mucosa should be considered :

a. 5% dehydrated
b. 10% dehydrated
c. 15% dehydrated
d. 20% dehydrated

17. A 2 year-old with fever is seen in hospital following a generalized seizure. He also has foul smelling, blood-tinged diarrhea. The culture of the stool is most likely to grow :

a. Campylobacter species
b. E.coli
c. Entamoeba histolytica
d. Salmonella species
e. Shigella species

18. Vomiting + Diarrhea for 3 days :

a. Rotavirus

19. Baby with buffy eyes and lower limb edema and protein +4 . He had upper respiratory tract infection before 2 weeks , investigation will show :

a. Low complement
b. High serum Na

c. Normal serum albumin
d. Normal fluid and electrolyte

20. Patient admitted with vomiting and diarrhea:

a. Rota virus

21. 6% dehydrated 20 kg patient . Calculate the dehydration deficit :

a. 1200
b. 1400
c. 1600
d. 1800
e. 2000

22. Which of the following is FALSE about dehydration ?

a. Increase elasticity of the skin

23. Baby has diarrhea for 3 days . What is the sign of dehydration ?

a. Tachycardia

24. Child with abdominal pain :

a. Reluctant to play means psychological cause
b. Guarding on examination is a serious sign
c. Vomiting is indicative for gastroenteritis
d. Radiological imaging is not advisable
e. Localized pain is indicative for non organic cause

25. Previously healthy 10 kg child has episode of gastroenteritis , irritable , lethargy and weighting 9 kg in ER , he is expected to has :

a. Mild to moderate dehydration
b. Elevated serum urea
c. Elevated calcium level
d. Decrease urea specific gravity

26. Which of the following is NOT a common finding in intussception?

a. Intermittent colicky pain.
b. Restlessness.
c. Vomiting.
d. Abdominal distension.
e. Anorexia.

27. The following are recognized complications of a hypernatremic dehydration EXCEPT :

a. Convulsions.
b. Cardiac arrhythmias.
c. Brain damage.
d. Metabolic acidosis.
e. Oliguria.

28. Regarding childhood gastroenteritis , all of the following are true EXCEPT :

a. Commonly caused by bacterial infections.
b. Oral rehydration solution (ORS) can be used in moderate dehydration.
c. Dehydration is a recognized complication.

d. Secondary lactose intolerance is a common complication.
e. Hemolytic – uremic syndrome may complicate some cases.

29. The most important indicator for severe dehydration is ?

a. Capillary refilling seen in 5 seconds
b. Sunken eye
c. Irritability

30. Celiac disease :

a. Hight is not affected
b. Steatorrhea at bearth
c. Shock syndrom is common with gluten challenge test
d. Normocytic normochromic anemia
e. Clubbing is an early sign

31. An 18 months old girl is brought by parents of evaluation of bloody diarrhea. You suspect an infectious etiology. Which of the following would NOT be included in the differential for bloody diarrhea?

a. Intussception.
b. Hemolytic uremic syndrome.
c. Hirschsprung's disease.
d. Formula allergy.
e. Bacterial enteritis.

32. Which of the following is not an expected laboratory finiding in a child with moderate hyponatremic dehydration :

a. Serum Sodium of 124 meq/L
b. Serum potassium 4.5 meq/L
c. Hematocrit is 28%
d. Random blood sugar 5 mmol/L

33. The true statement regarding acute gastroentritis in children is :

a. Pathogenic E.Coli is the commonest cause
b. Drowsiness out of the proportion to the degree of dehydration suggeests hypertonic dehydration
c. Marked sunken eyes and diminished skin turgor suggest 5% dehydration
d. Presence of seizures (convulsions) will exclude the diagnosis
e. Children should be kept at hospital until the stools have returned to normal

34. Causes if constipation in childhood include all of the following EXCEPT:

a. Hirschsprung's disease
b. Hypothyrodism
c. Spina bifida
d. Hypocalcemia
e. Gastroesophageal reflux

35. Chronic intractable diarrhea is caused by :

a. Rota virus
b. Cystic fibrosis
c. Immune deficiency

d. Celiac disease

36. Complication of viral gastroenteritis,except:

a. ARF
b. Hypertension
c. Metabolic acidosis
d. Hyponatremic dehydration

37. Which of the following support functional recurrent abdominal pain :

a. pain radiate to back
b. Prolonged fever
c. Periumbalical pain
d. Pain awake the child from sleep
e. Weight loss

38. The least important investigation in dehydration is:

a. CBC
b. ABG
c. Electrolyte
d. RFT
e. LFT

39. Uncommon cause of dehydration is:

a. Lasix
b. Burn
c. Nephrotic syndrome
d. Gastroenteritis

40. 16 kg boy , calculate maintenance IV fluid :

a. 1000 ml
b. 1200 ml
c. 1300 ml
d. 1600 ml

41. 30 kg dehydrated patient the maintenance is:

a. 1700

Answers

1)C	**2)D**	**3)B**	**4)E**	**5)A**	**6)C**
7)D	**8)B**	**9)E**	**10)E**	**11)B**	**12)E**
13)E	**14)B**	**15)E**	**16)B**	**17)E**	**18)A**
19)A	**20)A**	**21)A**	**22)A**	**23)A**	**24)E**
25)A	**26)B**	**27)E**	**28)B or A**	**29)A**	**30)E or D**
31)D	**32)C**	**33)B**	**34)D**	**35)??C**	**36)B**
37)C	**38)E**	**39)A**	**40)C**	**41)A**	

Liver & Spleen Disorders

1. **A healthy full-term, breastfed infant is jaundiced at 24 hours of age. The indirect serum bilirubin level is 210 umol/L. The mother is blood type O, Rh positive. Which of the following is most appropriate step :**
    a. Culture the blood
    b. Determine glucose-6-phosphate dehydrogenase activity
    c. Interrupt breast feeding
    d. Obtain a reticulocyte count
    e. Obtain blood type and Coombs test
2. **Which of the following is false about physiological jaundice ?**
    a. It rarely presents before the age of 24 hours
    b. It may cause kernicterus
    c. In premature infants it may persist for 3-4 weeks
    d. It is mainly due to temporarily impaired hepatic clearance of bilirubin
    e. It does not always require treatment with phototherapy.
3. **Conjugated hyperbilirubinemia is caused by :**
    a. Biliary atresia
4. **Regarding physiological jaundice , choose the correct answer ?**
    a. It appears in the 2nd day
5. **Differences between kidney and spleen include :**
    a. Kidney can go over it
    b. Notch on the lateral border
    c. Spleen resonant to percussion
6. **Kernicterus, where does it locate ?**
    a. Basal ganglia
7. **A boy infant after 12 hours of birth he developed jaundice and after 36 hours of age he had increase bilirubin level and reticulocytosis, what's your diagnosis?**
    a. SCA
    b. Thalassemia
    c. Rh incompatibility
    d. Polycythemia
    e. Doubin Johnsor
8. **First manifestation of kernicterus is:**
    a. Poor feeding and deep jaundice
    b. Week cry
    c. Convulsions
9. **Child from Chad came with hepatosplenomegaly. What is the best diagnostic test?**
    a. Blood film.
10. **Splenectomy is indicated in :**

Hereditary Spherocytosis.

11. **Cause of conjugated hyperbilirubinemia:**
    a. Galactosemia.
12. **Post splenectomy, increase risk of infection:**

a. Pnemoccous

13. Splenomegaly occurs in the following conditions EXCEPT:

a. Brucellosis
b. Malaria
c. Visceral Leishmaniasis
d. Congenital spherocytosis
e. G6PD

14. All of the following are case of indirect hyperbilirubinemia EXCEPT:

a. Deficiency of red cell enzymes
b. Hemoglobinopathy
c. Alpha-1 antitrypsin deficiency
d. Breast feeding
e. Ciggler-Najjar syndrome

15. All of the following are causes of hepatomegaly EXCEPT:

a. Glycogen storage disease type 1
b. Juvenile rheumatoid arthritis
c. Galactosemia
d. Heart failure
e. Physiological jaundice of the newborn

16. Regarding physiological jaundice in the newborn infant the true statement:

a. It presents in the first 24 hours
b. May be worsened by breast feeding
c. In premature infants, it may last for 4-6 weeks
d. Direct bilirubin levels may be as high as indirect bilirubin
e. It may cause kernicterus

17. Which of the following is the cause of continuous jaundice in 6 weeks infant:

a. Hypothyroidism
b. Physiological jaundice
c. Ciglar najjar syndrome
d. Wilson disease

18. All the following can cause hepatomegaly EXCEPT :

a. Hepatitis
b. Brucellosis
c. Liver cirrhosis

19. Jaundice with high direct bilirubin in the first month of life is about to be :

a. Biliary atresia

20. Differential diagnosis of jaundice in a 12 hours old infant includes all the following EXCEPT :

a. Sepsis.
b. Congenital spherocytosis.
c. Rh incompatibility.
d. Hypothyroidism.

e. Glucose-6-phosphate dehydrogenase (G6PD) deficiency.

Repeated Qs

21. All of the following is causes of indirect hyperbilirubinemia EXCEPT :

a. Deficiency of red cell enzymes
b. Hemoglobinopathy
c. Alpha-1 antitrypsin deficiency
d. Breast feeding
e. Criggler-Najjar syndrome

22. All of the following are causes of hepatomegaly EXCEPT :

a. Glycogen storage disease type 1
b. Juvenile rheumatoid arthritis
c. Galactosemia
d. Heart failure
e. Physiological jaundice of the new born

23. Regarding physiological jaundice in the newborn infant the true statement is

a. It presents in the firs 24 hours
b. May be worsened by breast feeding
c. In premature infants, it may last for 4-6 weeks
d. Direct bilirubin levels may be as high as indirect bilirubin
e. It may cause kernicterus

24. Splenomegaly occurs in the following conditions except :

a. Brucellosis
b. Malaria
c. Visceral leishmaniasis
d. Congenital sphercytosis
e. G6PD deficiency

25. All of the following are cause of splenomegaly except :

a. Brucella
b. Malaria
c. Visceral lishmaniasis
d. Congenital spherocytosis
e. G6PD deficiency

26. All of the following is causes of indirect hyperbilirubinemia except :

a. Deficiency of red cell enzymes
b. Hemoglobinopathy
c. Alpha-1 antitrypsin deficiency
d. Breast feeding
e. Criggler-Najjar syndrom

27. All of the following are causes of hepatomegaly except :

a. Glycogen storage disease type II
b. Juvenile rheumatoid arhtritis
c. Galactosemia
d. Heart failure
e. Physiological jaundice

28. Splenomegaly occurs in the following conditions EXCEPT:

a. Infectious mononucleosis
b. Malaria
c. In G6PD or cutaneous leshmania
d. Congenital spherocytosis
e. Thalassemia major

Answers

1)B	**2)B**	**3)A**	**4)A**	**5)A**	**6)A**
7)C	**8)A**	**9)A**	**10)A**	**11)A**	**12)A**
13)E	**14)C**	**15)E**	**16)C**	**17)C**	**18)C**
19)A	**20)D**	**21)C**	**22)E**	**23)C**	**24)E**
25)A	**26)C**	**27)E**	**28)C**		

Respiratory Disorders

1. **A 3 year old girl was brought to the emergency department by her mother with sudden onset of dyspnoea & paroxysmal cough. On examination her temperature was 7.6°C & she was having wheezing over right lung. The most likely diagnosis is:**
    a. Aspiration pneumonia
    b. Foreign body aspiration
    c. Acute epiglotitis
    d. Acute bronchiolotis
    e. Tension pneumothorax in the right lung
2. **In a child with symptoms characteristic of whooping cough the following are true EXCEPT:**
    a. Lymphocytosis
    b. The risk of brain damage is higher in infants less than 1 year
    c. Erythromycin treatment will completely resolve symptoms within 10 days
    d. Sub-conjunctival hemorrhage may be seen
    e. The infectivity period usually starts before the cough
3. **Common cause of pneumonia in first year of life include all EXCEPT:**
    a. H.influenzae
    b. Pneumococci
    c. RSV
    d. Mycoplasma pneumonia
    e. Staph. Aureus
4. **Croup:**
    a. Cased by bacterial agent
    b. Airway obstruction more commone in infants
    c. Most of cases are self limited
    d. Stridor is a prominent presenting sign
    e. Occur mostly during autumn
5. **D.Dx of wheezing in infants includes all of the following EXCEPT:**
    a. GER
    b. CF
    c. Trachioesphegeal fistula
    d. RT lobe collapse
    e. Bronchial asthma
6. **Commonest cause of acute bronchitis :**
    a. Adenoviruse
    b. RSV
    c. Parainfluenza
    d. Strept.pneumonia
    e. Mycoplasma pneumonia

7. **Which of the following is the true statement regarding viral bronchiolitis:**
    a. Parainfluenza virus is the commonest causative agent
    b. Inspiratory wheezes are characteristic
    c. Hyperinflated chest is a common clinical feature
    d. Never recurs after the 1st attack
    e. Bronchial asthma is not a differential diagnosis
8. **The true statement regarding infectious croup is:**
    a. Para influenza virus is commonly isolated
    b. Trismus is a recognized feature
    c. Laryngoscopy is necessary for diagnosis
    d. Corticosteroids are effective in relieving symptoms
    e. Is a common indication for tracheotomy
9. **In a child with pretussis the following statement are true EXCEPT:**
    a. Lymphocytosis is a feature
    b. Bronchopneumonia is the most frequent complication
    c. Young infants may have cough without the whoop
    d. Sub-sonjunctival hemorrhage may be seen
    e. The infectivity period starts in the paroxysmal stage
10. **A 6 month-old infant presents in the winter with fever , cough , wheezing , tachypnea and decreased appetite. A chest radiograph shows hyperaeration and streaky perihilar infiltrates bilaterally. You diagnose bronchopneumonia. Which organism would most likely be causing this child's infection ?**
    a. Chlamydia pneumoniae
    b. Mycoplasma pneumoniae
    c. Streptococcus pneumoniae
    d. Haemophilus influenzae
    e. Respiratory syncytial virus
11. **Which of the following investigations is most helpful in the assessment of a child presenting with an acute asthma attack ?**
    a. White cell count and differential
    b. Arterial blood gas
    c. Chest X-ray
    d. Pulmonary function test
    e. O2 saturation monitoring
12. **Which of the following is the most significant risk factor for the development of childhood asthma :**
    a. Family social background
    b. Parental asthma
    c. Stress in the family
    d. Parental smoking
    e. Presence of pets in the house

13. Which of the following steps is NOT indicated in the management of croup ?

a. Keeping the child calm
b. Hydration
c. Antipyretics
d. Antibiotics
e. Humidified oxygen

14. The most common cause of apnea in infants less than 6 months is :

a. Encephalitis
b. Seizure disorder
c. Cardiac arrhythmia
d. Milk allergy
e. Gastroesophageal reflux

15. The most common cause of hemoptysis in a 6 year-old boy is :

a. Cystic fibrosis
b. Pneumonia
c. Foreign body
d. Tracheobronchitis
e. Neoplasm

16. Which of the following is a characteristic chest x-ray appearance of respiratory distress syndrome (RDS) :

a. Air bronchograms
b. Fluid in fissures
c. Patchy infiltrate
d. Atelectasis
e. Enlarged heart

17. Severe asthma causes :

a. Ajauersis
b. One word senture
c. Silent chest
d. Respiratory flow rate < 50 %

18. Lobar pneumonia :

a. Normal movement in affected side
b. Normal mediastinal shifting
c. Normal breath sound
d. Decrease vocal resonance

19. Severity of asthma :

a. Less than 3 words in 1 breath
b. Silent chest
c. ICU admission
d. All of the above

20. Stridor , drooling saliva and high fever :

a. Epiglottitis

21. Diffuse wheezing with fine crepitation and hyper inflated lung ?

a. Bronchiolitis

22. One patient with stridor . What is the cause of the acute phase ?

a. Foreign body

23. Regarding bronchial asthma , choose the correct answer ?

a. Increase pCO2

24. About bronchopneumonia ?

a. RSV is the most common cause of it

25. True statement regarding pneumonia in children is :

a. In infancy , the cause is usually viral , particularly RSV

26. Wheezing is not a feature of :

a. Aspiration pneumonia
b. Bronchopulmonary dysplasia
c. Allergic croup
d. Bronchiolitis

27. In severe asthma :

a. Silent chest with no wheeze

28. Regarding mycoplasma pneumonia , which of the following is CORRECT :

a. Bilateral hilar lymph nodes in chest X-ray
b. Complicated with specific meningitis
c. Fever is the usual presentation

29. One of these can be seen in chronic upper airway obstruction :

a. Cor pulmonale
b. Pneumothorax
c. Asthma

30. The most common cause for acute bronchiolitis is :

a. Respiratory syncitial virus (RSV)

31. A 6 weeks old infant presents with a history of noisy breathing. The noise was first noted shortly after birth. It is inspirational in nature, worse now that the infant has a viral respiratory illness, the most likely diagnosis is :

a. Bronchial asthma.
b. Cystic fibrosis.
c. Laryngiomalacia.
d. Bronchopulmonary dysplasia.
e. Tuberculosis.

32. A 3 years old child has a positive tuberculin test. Which of the following would be most suggestive of military TB ?

a. Fever.
b. Hilar lymphadenopathy on chest X-ray.
c. Cough.
d. Elevated erythrocyte sedimentation rate.
e. Hepatosplenomegaly.

33. In croup, all the following are true EXCEPT:

a. Is caused by a bacterial agent.
b. Airway obstruction is more common.
c. In infants , most cases are self limited.
d. Stridor is prominent presenting sign.
e. Occurs mostly during autumn.

34. In a child with pertussis, the following statements are true except :

a. Lymphocytosis is a feature
b. ~~Bronchopneumonia is the most frequent complication~~
c. Young infants may have cough without the whoop
d. Sub-conjunctival hemorrahge may be seen
e. The infectivity period starts in the paroxysmal stage

35. A 3 year old girl presented to the ER with her parents Complaining from SOB and paroxysmal cough .. O/E : she was distresses with decrease air entry and wheezes in the Rt lung, the most likely diagnosis is :

a. Aspiration pneumonia
b. Acute epiglottitis
c. Foreign body in the Rt lung
d. Acute bronchiloitis
e. Tension pneumothorax of the Rt lung.

36. The following are X-ray finding in asthma except :

a. Cardiomegaly
b. Increased lung markings
c. Flat diaphragms
d. Increased lung lucency

37. A 9 year old child with fever, cough and crepitation, CXR shows infiltration and pleural effusion, the most likely organism is :

a. H.influenza is most likely and treated by acyclovir
b. Pneumococcal is the cause and can be treated by IV penicillin
c. Herpes virus
d. Chlamydia and treated by ceforoxime
e. RSV

38. A 5 month old, poor feeding and lethargy. O/E: signs of respiratory distress , wheezing and crepitation :

a. Parainfluenza maybe a cause

39. The true statement regarding infectious croup is :

a. Para influenza virus is commonly isolated
b. Trismus is recognized feature
c. Laryngogscoy is necessary for diagnosis
d. Corticosteroids are effective in relieving symptoms
e. Is a common indication for tracheotomy

40. In a child with primary pulmonary TB, the true statement is :

a. Diagnosis depends on finding acid-fast bacilli (AFB) is sputum
b. The mantoux test (PPD) is likely to be strongly positive
c. The child should be kept off school for the 1st sex months
d. The most likely source of infection is another child
e. Hemoptysis is the most predominant symptom

41. Which of the following is true regarding viral bronchiolitis :

a. Parainfluenza virus is the commenest causative cause
b. Inspiratory wheezes are characteristic
c. Hyperinflated chest is a common clinical feature
d. Never recures after the first attack
e. Bronchial asthma is not differential diagnosis

42. Wheezing is a feature of all the following EXCEPT :

a. Cystic fibrosis.
b. Congenital lobar emphysema.
c. Atypical pneumonia.
d. Gastroesophageal reflux.
e. Bronchiolitis.

43. Which of the following is a frequent infectious agent causing pneumonia in a 14 years old child with cystic fibrosis?

a. Staphylococcus aureus.
b. Nisseria gonorrhea.
c. Pseudomonas aerogenosa.
d. Klebsiella pneumonia.
e. Streptococcus pneumonia.

44. Wheezing may be seen in a child with all except :

a. Inhaled foreign body
b. Aspiration of gastric contents
c. Epiglottis
d. Bronchial asthma

45. Dull percussion note on the chest maybe due to all the following except :

a. Hemothorax
b. Lung collapse
c. Emphysema
d. Consolidation
e. Pleural effusion

46. In pertussis infection, all is true except :

a. Lymphocytosis
b. Bronchopneumonia is a common complication
c. Young infants have cough without a whoop
d. Subconjuctival hemorrahge maybe seen
e. Infection period star during the paroxysmal stage

47. In a child with primary pulmonary TB, the statement is:

a. Diagnosis depends on finding acid-fast bacilli (AFB) in sputum
b. The mantoux test (PPD) is likely to be strongly positive
c. The child should be kept off school for the 1st 6 months
d. The most likely source of infection is another child
e. Hemoptysis is the most predominant symptom

48. Acute severe asthma :

a. Silent chest.

49. Bronchiolitis oblitrans complicated by:

a. Adenovirus

50. Cause of Empyema :

a. Streptococcus Pneumoniae.
b. streptcococcus varidans.

51. 1st line management in croup (Epiglottis) is:

a. Secure airway

52. Regarding severe acute asthma , all are true EXCEPT :

a. Tachypnea
b. Tachycardia
c. Loud voice wheezing
d. Peak flow < 50%
e. Cannot talk

53. Best investigation in patient with recurrent wheezing, nasal polyp, malabsorption, impaired LFT:

a. Bronchography
b. ABG
c. Sweat chloride test
d. Lung scan
e. PFT

54. In hyperventilation:

a. ↑ pH, ↑ PCO2
b. ↓ pH, ↓ PCO2
c. ↓ pH, ↑ PCO2
d. ↑ pH, ↓ PCO2

55. Primary TB can occur in all the following sites EXCEPT :

a. Skin
b. Brain
c. Gut
d. Lung
e. Lymph node

56. All cause otitis media EXCEPT :

a. Group A β-hemolytic streptococci
b. Pseudomonas
c. Hemophilus
d. Moroxella

57. 6 month old infant is considered to be tachypneic if his respiratory rate is

a. 20 / minute
b. 30 / minute
c. 40 / minute
d. 50 / minute
e. 60 / minute

58. All are true about bronchiolitis EXCEPT :

a. It is mostly caused by adenovirus

59. All are in favor of epiglottitis rather than viral croup EXCEPT :

a. Harsh , barking cough

60. Wheezing can occur in all EXCEPT:

a. Atypical pneumonia

61. Patient in respiratory distress with right tracheal deviation the chest X-ray will be:

a. Lung collapse

62. Barking cough at night:

a. Spasmodic croup.

63. The most com cause of bronchiolitis is:

a. RSV

64. Not a feature of TB on chest x-ray:

a. Subpleural infiltrate

65. The commonest cause of pneumonia in 6 year old child is:

a. Streptococcus pneumonia

Answers

1)B	**2)B**	**3)D**	**4)D**	**5)D**	**6)E**
7)C	**8)A**	**9)??C**	**10)E**	**11)E**	**12)B**
13)D	**14)E**	**15)B**	**16)A**	**17)D**	**18)B**
19)D	**20)A**	**21)A**	**22)A**	**23)A**	**24)A**
25)A	**26)C**	**27)A**	**28)C**	**29)A**	**30)A**
31)C	**32)??E**	**33)A**	**34)C**	**35)C**	**36)A**
37)B	**38)A**	**39)A**	**40)A**	**41)C**	**42)B**
43)C	**44)C**	**45)C**	**46)C**	**47)A**	**48)A**
49)A	**50)A**	**51)A**	**52)E**	**53)C**	**54)D**
55)??B	**56)B**	**57)D**	**58)A**	**59)A**	**60)A**
61)A	**62)A**	**63)A**	**64)A**	**65)A**	

Cardiac Disorders

1. **All are true about innocent murmur EXCEPT:**
    a. Commonly change with position
    b. Extends to diastole
    c. Not radiated to the back
    d. Short duration
    e. Not localized to the apex
2. **Right ventricular hypertrophy is seen in all of the following EXCEPT:**
    a. PDA
    b. Pulmonary stenosis
    c. ASD
    d. Tetralogy of Fallot
3. **Recognized cause of hypertension in childhood include all of the following EXCEPT:**
    a. Chronic glomerulonephritis
    b. Hypernatremic dehydration
    c. Renal vein thrombosis
    d. Congenital adrenal hyperplasia
    e. Coarctation of the aorta
4. **Regarding left to right shunt which is NOT true:**
    a. ECG findings in PDA show right ventricular hypertrophy
    b. In ASD there is right ventricular hypertrophy
5. **All are present in infective endocarditis EXCEPT:**
    a. Roth's spot
    b. Splenomegaly
    c. Petechia
    d. Strept. Viridians is the commonest organism
    e. Blood culture is +ve
6. **All of the following can cause deviation of apex beat to the right EXCEPT:**
    a. Pleural effusion on the left side of the chest
    b. Lobar pneumonia on the left side of the chest
    c. Tension pneumothorax in the left side of the chest
    d. Collapse of the right lung
    e. Fibrosis of the right lung
7. **Which of the following is true regarding cyanotic heart disease:**
    a. Infants commonly are severely distressed in proportion to the degree of cyanosis
    b. There is usually an increased risk of cerebral abscess
    c. Clubbing is usually present at birth
    d. In an infant with tetralogy of Fallot's the chest x-ray characteristically shows pulmonary plethora
    e. Ventricular septal defect (VSD) is a common cause

8. **The true statement regarding ventricular septal defect (VSD)is:**
    a. The smallest defects tend to produce the softest murmurs
    b. Left to right shunting leads to increase the risk of cerebral abscess
    c. The characteristic murmur is a pansystolic murmur at the left sternal border
    d. The systolic murmur increase in intensity as pulmonary hypertension develops
    e. There is no risk of bacterial endocarditis
9. **The following of substernal heave in palpation of the precordium is most likely associated with which of the following?**
    a. Left ventricular hypertrophy
    b. An ejection click
    c. Systolic hypertension
    d. Right ventricular hypertrophy
    e. Pericardial effusion
10. **Common signs of heart failure in children include all of the following EXCEPT :**
    a. Decreased exercise tolerance
    b. Fatigue
    c. Failure to thrive
    d. Frequent chest infections
    e. Lower limb edema

11. **Which of the following is NOT characteristics of functional murmur ?**
    a. Pansystolic murmur
    b. Murmur varies with position
    c. Variably split S2
    d. Murmur becomes louder with fever
    e. No extra clicks
12. **Which of the following is the most congenital heart lesion in children ?**
    a. ASD
    b. VSD
    c. PDA
    d. TOF
    e. Coarctation of the aorta
13. **Squatting position with episodes of cyanosis :**
    a. Cardiomyopathy
    b. VSD
    c. TOF
    d. Tracheoesophageal fistula
14. **Regarding murmurs , which of the following is correct :**
    a. Continuous murmur on the left infra clavicular region → PDA

15. Choose the right answer ?

a. ASD is associated with splitting of the 2nd heart sound

16. An early manifestation of newborn heart failure is ?

a. Poor feeding

17. Which of the following is not a feature of innocent murmur ?

a. Positive thrill

18. All the following are signs of heart failure in infants EXCEPT :

a. Poor feeding
b. Sweating
c. Tachypnea
d. Tachycardia
e. Lower limb swelling

19. Accentuated strong 2nd heart sound is seen in ?

a. Aortic stenosis
b. ASD
c. VSD
d. Teratology of fallot

20. Plethoric lung is seen in :

a. Truncus arteriosus
b. Pulmonary stenosis
c. PDA

21. One of these will not cause infective endocarditis :

a. ASD
b. VSD

22. Which of the following statements concerning auscultation of the heart in children is FALSE ?

a. The second heart sound is normally split.
b. The third heart sound maybe normal.
c. The first heart sound is due to closure of the atrioventricular valve.
d. The second heart sound is due to closure of the semilunar valve.
e. The first heart sound is normally split.

23. ECG changes suggestive of hyperkalemia include which of the following ?

a. Prolonged QT interval.
b. Prolonged PR interval.
c. T wave inversion.
d. Bradycardia.
e. ST segment depression.

24. All of the following are associated with teratology of Fallot EXCEPT :

a. Cerebral hemorrhage.
b. Cyanotic spells.
c. Cerebral abscess.
d. Single second heart sound.
e. Squatting.

25. Major criteria of rheumatic fever is :

a. Fever
b. Prolonged PR interval
c. Arthritis
d. Raised ESR

26. Which of the following is true regarding the carditis of acute rheumatic fever?

a. Is characterized by valve involvement.
b. If asymptomatic during the acute phase , valvular insufficiency will not occur.
c. Aortic insufficiency is the most common valvular abnormality.
d. No change in incidence of valve involvement if treated early.
e. ECHO is not useful in the work up .

27. The following are true regarding rheumatic fever EXCEPT :

a. Migrating arthritis is one of the major criteria
b. Usually preceded by pneumonia 2-6 weeks previously
c. Show a sustained rise of the Anti-streptolysin O titer
d. May have systolic cardiac murmur
e. Usually occur in children of school age

28. Patient known to have Rheumatic heart disease, presented with fever for 2 weeks :

a Infective endocarditis

29. Cyanotic heart disease :

a. VSD is a common cause
b. TOF is a common cause of heart failure in early infancy
c. Cavernous sinus thrombosis is one of the complication
d. TOF is associated with increase pulmonary hypertension
e. Clubbing is present at birth

30. VSD :

a. Diastolic murmur at apex
b. Pulmonary hypertension is associated with increase the intensity of the murmur
c. Samllest gab, the softest is the murmur
d. Endocarditis doesn't occur.
e. Cerebral abscess is a complication

31. Signs of heart failure in infancy include all of the following except :

a. Tachycardia
b. Cyanosis
c. Splenomegaly
d. Hepatomegaly
e. Poor weight gain

32. Each of the following is a major criteria for R.F EXCEPT:

a. Carditis
b. Sydenhams's chorea
c. Subcutaneous nodules
d. Erythema marginatum
e. Fever

33. Associated with rheumatic chorea:

a. Fine tremor with rest
b. Convulsions
c. ↑ motor strength
d. Irregular jerky movements
e. Muscle fibrillation

34. PDA closes by:

a. Prostaglandin
b. Anti-prostaglandin
c. Estrogen
d. Anti-estrogen
e. Progesterone

35. Enlarged right atrium & ventricle in X-ray:

a. Tricuspid stenosis
b. Aortic stenosis
c. PDA
d. Tricuspid regurgitation
e. Mitral stenosis

36. All of the following characteristics of murmur is significant EXCEPT :

a. Radiated over the pericardium
b. Soft , musical murmur
c. Thrill
d. Diastolic murmur
e. Associated with other cardiac signs

37. Doesn't cause parasternal heave:

a. PDA.

38. Which of the following is sign of heart failure:

a. Hepatomegaly

39. Which of the following is a cyanotic heart disease:

a. Tricuspid atresia

40. The most diagnostic criteria in rheumatic heart is:

a. Chorea

41. 8 days baby, cry, bluish :

a. Tetralogy of Fallot.

42. All with hypoxemia EXCEPT :

a. Left to right shunt.

43. Oligemic lung field :

a. Teratology of Fallot .

Answers

1)B	**2)A**	**3)E**	**4)A**	**5)E**	**6)B**
7)B	**8)C**	**9)D**	**10)E**	**11)A**	**12)B**
13)C	**14)A**	**15)A**	**16)A**	**17)A**	**18)E**
19)A	**20)C**	**21)A**	**22)E**	**23)B**	**24)?**
25)B	**26)A**	**27)B**	**28)A**	**29)B**	**30)A**
31)C	**32)E**	**33)D**	**34)B**	**35)D**	**36)B**
37)A	**38)A**	**39)A**	**40)A**	**41)A**	**42)A**
43)A					

Kidney

&

Urinary Tract

1. **Regarding nephritic syndrome, all the following are correct EXCEPT:**
    a. Hypoaldosteronism
    b. Decrease intravascular volume
    c. Increase cholesterol
    d. Low heamtocrit
2. **Complication of nephritic syndrome include all EXCEPT:**
    a. Acute renal failure
    b. Bacterial peritonitis
    c. Hypocoagulable state
    d. Renal vein thrombosis
    e. hypertension
3. **About acute renal failure all true EXCEPT:**
    a. Low urine output
    b. low serum protein
    c. dehydration is a recognizaed cause
    d. post streptococcal GN is a cause
    e. may present with pulmonary edema
4. **Post. Streptococcal GN include all the following EXCEPT:**
    a. Oligouria
    b. Normal IgA level
    c. Decreased complement C3 level
    d. Dysuria
    e. Presence of granulocytes and RBCs casts
5. **Which of the following is true regarding post streptococcal glomerulonephritis?**
    a. Long term prophylaxixs with penicillin is indicated to prevent recurrence
    b. There's usually associated generalized edema
    c. Heavy proteinuria is usually present
    d. Usually presents with microscopic hematuria
    e. Most cases recover completely
6. **An 8 year-old boy who has been continent of urine between the ages of 4 and 5 years comes in because of recent onset of enuresis. Which of the following statements is FALSE ?**
    a. It may be precipitated by a family crisis
    b. It does not require a specific treatment
    c. It can lead to poor self esteem
    d. It responds best to individual psychotherapy
    e. It can be secondary to diseases such as diabetes mellitus
7. **The most likely diagnosis in a 3 month-old with a left flank mass is :**
    a. Renal cell carcinoma
    b. Wilm's tumor
    c. Neuroblastoma
    d. Renal stones
    e. Polycystic kidney disease

8. **UTI :**
    a. Vesicoureter reflux
    b. Chronic diarrhea
9. **Baby with puffiness of the eyes and lower limb edema and protein +4 and he has upper respiratory tract infection before 2 weeks. Which is the most likely diagnosis ?**
    a. Nephrotic syndrome
10. **Nephrotic syndrome in infants which is true :**
    a. Hematuria
    b. Hypoalbuminuria
    c. Hyperlipidemia
    d. Hypertension
    e. Hyponatremia
11. **Which of the following is true regarding post streptococcal glomerulonephritis :**
    a. Long term prophylaxis with penicillin is indicated to prevent recurrence
    b. There is usually associated generalized edema
    c. Heavy proteinurea is usually present
    d. Usually presents with microscopic hematuria
    e. Most cases recover completely
12. **Causes of nephrotic syndrome include all of the following EXCEPT :**

a. Congenital syphilis.
b. Group B streptococci.
c. SLE.
d. Gold therapy.
e. Malaria.

13. **In acute renal failure , which of the following is true :**
    a. Hyperkalemia
    b. Metabolic alkalosis
    c. Metabolic acidosis
14. **Which of the following is true regarding post streptococcal glomerulonephritis :**
    a. Long term prophylaxis with penicillin is indicated to prevent recurrence
    b. There is usually associated generalized edema
    c. Heavy proteinurea is usually present
    d. Usually presents with microscopic hematuria
    e. Most cases recover completely
15. **Complications of post strept.GN include all the following except :**
    a. Hyperkalemia
    b. Hypernatremia
    c. Pulmonary edema
    d. Encephalopaty
    e. Anemia
16. **All causes proteinuria except:**

a. Exercise
b. Cold
c. Fever
d. UTI
e. Hypokalemia

17. Post streptococcus glomerulonephritis:

a. Normal IgA
b. Low complement level
c. Granular and red cell cast
d. Oliguria
e. Dysuria

18. In IgA nephropathy, complement to be decreased:

a. C1
b. C2
c. C3
d. C4
e. C5

19. Regarding minimal change nephrotic syndrome , all are true EXCEPT :

a. Normal blood pressure
b. Low complement C3
c. No microscopic hematuria
d. Normal renal function
e. Age from 1 to 10 years

20. Common finding in CRF:

a. Growth retardation
b. Abdominal pain
c. MR

21. Which of the following in nephrotic syndrome:

a. ↓ Hematocrit.
b. ↓ Osmolality
c. Hypoalosteronemia

22. The cause of post infection glomerulonephritis:

a. Group A Streptococcus.

23. In steroid dependant nephrotic syndrome:

a. The complement is normal

Answers

1)A	**2)C**	**3)A**	**4)B**	**5)?**	**6)?**
7)?	**8)?**	**9)A**	**10)?**	**11)?**	**12)?**
13)C	**14)?**	**15)B**	**16)?**	**17)?**	**18)C**
19)B	**20)?**	**21)?**	**22)?**	**23)?**	

Hematological Disorders

1. **All of the following is true about sickle cell anemia EXCEPT:**
    a. Normal immunity
    b. Hand foot syndrome
    c. Osteomyelitis is a complication
    d. If the CNS is affected the child need exchange transfusion
2. **All of the following are causes of Microcytic hypochromic anemia EXCEPT:**
    a. Iron deficiency anemia
    b. Thalassemia
    c. Folic acid deficiency
    d. Sideroblastic anemia
3. **All the following is true in hemophilia EXCEPT:**
    a. Prolonged PTT
    b. Hemarthrosis
    c. Affects male and female equally
    d. X-linked
4. **The following are constituents of von-willebrand disease EXCEPT:**
    a. Presence of hemarthrosis
    b. Presence of bleeding
    c. X-linked inheritance
    d. Prolonged APTT
    e. Prolonged bleeding time
5. **Which of the following is the cause of normocytic normochromic anemia:**
    a. Iron deficiency due to cow's milk protein intolerance
    b. Chronic renal failure
    c. Thalassemia major
    d. Pernicious anemia
    e. Folate deficiency
6. **All of the following are features of hemophilia A EXCEPT:**
    a. Males & females are equally affected
    b. Hemarthrosis can be the presenting feature
    c. Activated partial thromboplastin time (APTT) is always prolonged
    d. Patients are prone to acquire hepatitis C
    e. Patient with factor VIII less than 1% may have spontaneous bleeding
7. **Which of the following statements about sickle cell disease is true ?**
    a. All patients with sickle cell disease have a homozygous HbSS
    b. Sickle cell disease causes a severe chronic anemia that is not routinely transfusion dependent .
    c. Patients have increased susceptibility to infection by non-encapsulated organisms
    d. Patients usually present with sickle cell crisis within one month of age

e. Splenic dysfunction usually does not occur until the child enters his/her teens

8. **Sickle cell anemia exchange transfusion :**
    a. Aplastic crisis
    b. Hyperhemolytic crisis
    c. Hypersplenism
    d. Autosplenectomy
    e. Priapism
9. **Iron deficiency anemia :**
    a. Increased reticulocytes
    b. Low serum transferrin
    c. Spherocytes in peripheral blood
10. **Bleeding after circumcision and brother with bleeding disorder :**
    a. Factor VIII
11. **patient with swelling , jaundice and pallor. Which kind of anemia he has ?**
    a. Normochromic normocytic anemia
12. **Liver edge palpable in all except ?**
    a. Sickle cell anemia
13. **Leucocytosis is a diagnostic feature of what ?**
    a. Leukemia
14. **Choose the correct answer ?**
    a. Macrocytic anemia is associated with folic acid deficiency
15. **1 month old baby , he has bleeding after circumcision and his old brother have the same condition . What is the problem ?**
    a. Prolonged PTT
16. **Vaso-occlusive crisis of sickle cell disease can be differentiated from osteomylitis by :**
    a. Fever
    b. MRI finding
    c. Swelling
    d. Tenderness
    e. High ESR
17. **Prolonged bleeding time is not seen in :**
    a. DIC
    b. Von Willibrand's disease
    c. Factor 8 deficiency (hemophilia A)
18. **Which is correct about thalassemia :**
    a. Do not need regular transfusion
    b. Absent globin chain
    c. Increase iron load
19. **Regarding idiopathic thrombocytopenic purpura (ITP) , all the following are true EXCEPT :**

a. Purpuric rash is typically over the extensor surface of the limbs .
b. Bone marrow examination reveals increased megakaryocytes .

c. Intracranial hemorrhage is a recognized complication.
d. Steroid is one of the modes of therapy.
e. May be proceeded by infection .

20. One of these is seen in congenital hemolytic jaundice :

a. High level of stercobilinogen

21. Regarding sickle cell anemia, all of the following are true EXCEPT :

a. Priapism is an indication for exchange transfusion.
b. Loss of the ability to concentrate the urine is a recognized complication.
c. Screening should be done for the whole family.
d. Patients are liable to develop sepsis with encapsulated organisms.
e. Patients should receive yearly pneumococcal vaccine.

22. Iron deficiency anemia is characterized by all of the following EXCEPT :

a. Low iron stores.
b. High total iron binding capacity.
c. Electrophoresis is the diagnostic test.
d. Microcytic hypochromic picture in the blood film.
e. Free erythrocyte protoporphyrin is increased.

23. Which of the following is true regarding hemophilia ?

a. It is autosomal dominant disorder.
b. Factor 9 deficiency is also known as hemophilia B.
c. In hemophilia A , prothrombin time and bleeding time are both abnormal.
d. They have Hepatosplenomegaly.
e. HIV transmission is common in hemophiliac.

24. 1 ½ year old, microcytic hypochromic anemia, Hb:6.5 Retic 2%, the first line of management is :

a. IM Iron sulfate
b. Oral iron
c. Blood transfusion

25. One of the following does not support the chemical findings of thalassemia :

a. Target cells in the peripheral blood
b. Hb less than 5gm/dl
c. Decrease serum ferritin
d. Hypochromatic cells
e. Increase HbF

26. Iron deficiency anemia is characterized by all of the following except :

a. Decreased serum iron
b. Increased iron reabsorption from the gut
c. Decrease ferritin
d. Decrease iron binding capacity
e. Decrease MCV

27. About SCA all is true EXCEPT :

a. Diagnosed by HB electrophoresis
b. Hand and foot syndrome is usually the 1st presentation
c. Aplastic crises is the most common crises
d. Increased risk of infection
e. Cardiomegay is a common finding in older children

28. Thrombocytopenia present in all of the following except :

a. ITP
b. Henoch-Schonlein purpura
c. SLE
d. Leukemia
e. Aplastic anemia

29. The true statement about SCA is :

a. Positive SC test is diagnostic
b. Heterozygous is asymptomatic
c. Treatment with desferoxamine improve the life expectancey
d. Hand-foot syndrom commonly occurs after 5 year age
e. Splenomegaly is usual finding in a 10 year child SCA

30. To differentiate between vaso-occlusive crises and osteomyelitis in SCA, most helpful tool is:

a. ESR
b. CBC
c. Blood culture
d. Isotope bone scan
e. Hb electrophoresis

31. Why in SCA there is high risk of infections?

a. Defect in opsonization

32. Von Willibrand disease:

a. Long bleeding time

33. Indication of blood transfusion in SCA is:

a. Sequestration crisis

34. Bleeding in Henoch-Schonlein is due to:

a. Vasculitis

35. Hb electrophoresis in thalassemia trait :

a. HbA2.

36. About iron deficiency anemia , all are true EXCEPT :

a. blood loss is a common cause in infant
b. increase RDW
c. Pica

37. Which of the following is true regarding β-thalassemia :

a. Often treated by bone marrow transplantation
b. Present with gallstone
c. Treated by using folic acid and iron
d. β – trait usually symptomatic

38. Regarding hemophilia A , all are true EXCEPT :

a. X-linked
b. Severity is constant in family
c. Spontaneous bleeding in joint and muscle
d. Increase factor VIII 30% before surgery is significant

39. In HUS (hemolytic uremic syndrome):

a. Thrombocytopenia
b. Hematuria
c. ARF
d. +ve comb test

40. Regarding ITP :

a. Caused by production of anti-platelets antibodies
b. Has high risk of intracranial hemorrhage
c. Relapses and remits over several years
d. Can be treated by platelet transfusion
e. Hemoartherosis and bleeding after surgery

41. 12 year old SCA, all present except:

a. Splenomegaly

Answers

1)A	**2)C**	**3)C**	**4)C**	**5)B**	**6)A**
7)C	**8)E**	**9)B**	**10)A**	**11)A**	**12)A**
13)A	**14)A**	**15)A**	**16)E**	**17)B**	**18)B or C**
19)?	**20)A**	**21)?**	**22)C**	**23)B or C**	**24)C**
25)C	**26)D**	**27)C**	**28)B**	**29)?**	**30)E**
31)A	**32)A**	**33)A**	**34)A**	**35)A**	**36)A**
37)B	**38)?**	**39)?**	**40)A**		

Neurological Disorders

1. **Which statement is true regarding Bell's palsy:**
    a. Only upper part of the face is affected
    b. Mouth will be deviated toward the affected side
    c. It is LMNL
    d. Is not reversible
    e. Usually other cranial nerves are affected
2. **The followings are typical of simple febrile convulsion :**
    a. Positive family history
    b. Postictal paralysis
    c. Onset between 6 months and 6 years
    d. Rapid rise of temperature
    e. Lasts less than 15 minutes
3. **Regarding meningitis all true EXCEPT:**
    a. Latex agglutination test is diagnostic in viral meningitis
    b. Gram stain will show intracellular diplococcic in Niesseria meningitis
    c. CSF glucose of less than 40 of blood glucose is suggestive of bacterial cause
    d. GBS is common organism in neonatal period
    e. Papilloedema is a contraindication to lumber puncture
4. **All of the following cause peripheral neuropathy EXCEPT:**
    a. Post infectious polyneuropathy
    b. Lead poisoning
    c. DM
    d. Spinal muscular atrophy
5. **In cerebral palsy the true statement is:**
    a. Hypotonia with decrease deep tendon reflex
    b. Progressive
    c. Mental retardation associated up to 90%
    d. Persistent neonatal reflex
    e. Choreoathetosis is commonest
6. **All are signes of LMNL EXCEPT:**
    a. Flaccid paralysis
    b. Fasciculation
    c. +ve babiniski sign
    d. Decrease tendon reflexes
    e. Hypotonia
7. **Case of CP:**
    a. Birth injury
    b. Congenital infection
    c. Prematurity
    d. Degenerative brain disease
    e. Neonatal hypoglyceamia

8. **Complications of acute bacterial meningitis include all of the following EXCEPT:**
    a. Inappropriate secretion of ADH
    b. Extradural effusion
    c. Convulsions
    d. Cranial nerve palsies
    e. Hydrocephalus
9. **Which of the following is TRUE regarding cerebral palsy?**
    a. The cause is almost always identifiable
    b. Spasticity is present at birth
    c. Sensory loss is present in most cases
    d. Severe indirect hyperbilirubinemia can be a cause
    e. Pertussis vaccine is contraindicated
10. **The TRUE statement regarding febrile seizures is :**
    a. They are usually foal
    b. They are recurrent in 85% of cases
    c. Characteristically occurs in children age 6 month to 5 years
    d. Usually Mendelian type of inheritance
    e. They have a poor prognosis
11. **Causes of Floppy infant include all the following EXCEPT:**
    a. Maternal myasthenia gravis
    b. Down syndrome
    c. Ricjets
    d. Werding Hoffmann disease
    e. Ducheinne muscular
12. **The viral infection most likely to cause CNS involvement and focal neurological findings is :**
    a. Coxackievirus
    b. Herpes simplex
    c. Enterovirus
    d. Rabies
    e. Rhinovirus
13. **A 15 month-old child has spastic cerebral palsy. Which of the following is most suggestive of perinatal asphyxia as the cause for this condition :**
    a. Evidence of periventricular calcification at birth
    b. Microcephaly at birth
    c. Recurrent seizure in the newborn period
    d. Retinopathy of prematurity
    e. Rigidity and spasticity
14. **CP :**
    a. Feeding difficulty
    b. Diplegia upper limb affected more than lower limb
    c. Hemiplegia is the most common

15. Central hypotonia :

a. Preserved primitive reflexes
b. Normal mentality

16. Complication of bacterial meningitis :

a. Hydrocephalus

17. 6 months infant presented with high fever , convulsions , CSF gram stained showed gram negative coccobacilli . What is the organism ?

a. H.influanza
b. E.coli

18. Newborn with generalized hypotonia ?

a. Spinal muscle atrophy

19. Case of febrile seizure which of the following is not a typical feature of it ?

a. Neck rigidity

20. All the following are true about cerebral palsy (CP) EXCEPT :

a. Spastic type is the commonest
b. Kernicterus associated with extrapyramidal lesion
c. The most common cause is idiopathic
d. Progressive loss of milestone is a typical presentation
e. Sensory involvement typically not present

21. Lesion of which of the following cranial nerves can cause horizontal diplopia :

a. Optic nerve
b. Trochlear nerve
c. Abducent nerve
d. Sympathetic chain
e. Facial nerve

22. One of these is true about cerebral palsy ?

a. Sensory involvement is highly common
b. Obscure cause is mostly seen

23. Anterior horn disease can cause :

a. Sensory loss
b. Fasciculation

24. What is the underlying brain injury that occur in peripartum period resulting in cerebral palsy ?

a. Cerebral edema.
b. Interventricular bleeding.
c. Hypoxic-ischemic encephalopathy.
d. Hyper osmolality.
e. Hypopituitarism.

25. What is the most common cause of hydrocephalus ?

a. Reduced absorption of CSF .
b. Excess production of CSF.
c. Obstruction of CSF.
d. Anencephaly.
e. Birth trauma.

26. Regarding febrile seizure, all of the following are true EXCEPT :

a. Onset between 6 months and 6 years.
b. Seizures lasting less than 15 minutes.
c. Positive family history.
d. Seizure are more likely to occur with the rapid rise of temperature.
e. Lumbar puncture is diagnostic.

27. The following are signs of lower motor neuron lesion EXCEPT :

a. Decreased deep tendon reflexes
b. Hypotonia
c. +ve Babiniski sign
d. Fasiculations
e. Flaccid posture

28. Complications of acute bacterial meningitis include all of the following except

a. Inappropriate secrestion of ADH
b. Extradural effusion
c. Convulsions
d. Cranial nerve palsies
e. Hydrocephalus

29. Which of the following is true regarding cerebral palsy :

a. The cause is almost always identifiable
b. Spasticity is present at birth
c. Sensory loss is present in most cases
d. Sever indirect hyperbilirubinemia can be a cause
e. Pertussis vaccine is contraindicated

30. The true statement regarding febrile seizures is :

a. They are usually focal
b. They are recurrent in 85% of cases
c. Characterisitically occures in children age 6 months to 5 years
d. Usually of Mendelian type of inheritance
e. They have a poor prognosis

31. Causes of floppy infant include all of the following except :

a. Maternal myasthenia gravis
b. Down syndrom
c. Rickets
d. Werding hoffmann disease
e. Ducheinne muscular

32. Cerebellar disease characterized by all of the following except :

a. Ataxia
b. Diminished deep tendon reflex
c. Hypotonya
d. Resting tremor
e. Dysarthria

33. Complications of acute bacterial meningitis include all except :

a. Inadequate secretion of ADH
b. Convulsions
c. Hypoglycemia
d. Hydrocephalus
e. Cranial nerve palsies

34. Causes of cerebral palsy include all of the following except :

a. Hypoglycemia
b. Hyperbilirubinemia
c. Congenital infections
d. Hypoxya
e. Congenital hypothyroidism

35. All causes peripheral hypotonia except :

a. Spinal muscular dystrophy
b. Down syndrom
c. Poliomyelitis
d. Myasthenia gravis
e. Myotonic dystrophy

36. Commonest type of cerebral palsy is :

a. Hypotonic
b. Athetotic
c. Spastic
d. Mixed
e. Ataxic

37. According to febrile convulsions :

a. Usually focal
b. Recurrence in 85% of cases
c. Occurs in age from 6 months to 5 years
d. Has bad prognosis

38. In septic meningitis all is true except :

a. Increase glucose in CSF
b. Decrease glucose
c. Increase WBC
d. Increae protein

39. Anterior horn cell disease is :

a. Myasthenia graves
b. Gillian barre's syndrom
c. Warding hoffman's
d. Chorea
e. Atonic dysplegia

40. Regarding Bell's palsy , the TRUE statement is :

a. Only the upper part of the face is affected.
b. Mouth will be deviated toward the affected side.
c. It is a lower motor neuron lesion.
d. It is irreversible.
e. Usually other cranial nerves are affected.

41. Causes of floppy infant include all the following EXCEPT :

a. Maternal myasthenia gravis.
b. Down's syndrome.
c. Rickets.
d. Werdnig Hoffmann disease.
e. Duchenne muscular dystrophy.

42. 3 months infant with meningitis is likely to have :

a. Rash
b. Stiff neck
c. Bulging anterior fontanelle
d. Increase CSF sugar
e. Decrease CSF proteins

43. Meningitis cause all of the following EXCEPT :

a. Learning disability
b. Deafness
c. Blindness
d. Cataract

44. 7 year old patient had new seizure. 2 years back he had measles:

a. Cerebral abscess
b. SSP (Subacute Sclerosing Panencephalitis)
c. Subdural effusion
d. Benign febrile seizure
e. SAH (subarachnoid hemorrhage)

45. Weak knee jerk, lesion at:

a. L3 – L4
b. L5
c. L5 – S1
d. S1 – S2
e. S2 – S3

46. Seizure in neonate of diabetic mother:

a. Hyponatremia
b. Hypoglycemia
c. Hypophosphatemia
d. CNS infection
e. Hypokalemia

47. Regarding CP , all of the following are true EXCEPT :

a. Primitive reflexes are present
b. Progressive loss of milestone
c. Kernicterus cause extrapyramidal type
d. Spastic is the most common

48. All of the following can cause meningitis in children EXCEPT :

a. Group B streptococci
b. Nisseria meningitides
c. Hemophilus influenza

49. Child with URTI treated with antibiotic after that he become lethargic (glucose: normal, protein: 50, culture +ve), what's your diagnosis?

a. Septic meningitis
b. Aseptic meningitis
c. Partial treated meningitis

50. Cause of meningitis and sepsis in neonates :

a. E. Coli
b. Listeria monocytogens

51. The difference between complex and simple seizure :

a. loss of consciousness in complex seizure

52. The difference between CP and degenerative brain disease :

a. CP is not progressive.

53. Tongue Fasciculation :

a. Spinal muscular atrophy.

54. Pupil constriction:

a. Cranial nerve III lesion.

55. Poliomyelitis is often misdiagnsoed with:

a. Guillain-Barre syndrome .

56. In absence seizures:

a. The EEG finding is 3 seconds spikes

57. Cerebral palsy case. What will help to exclude in differential diagnosis:

a. Sensory involvement

58. Injury to C6 causes weakness to:

a. Biceps.

59. Most area affected in HSV encephalitis:

a. Temporal area.

60. Case: tongue facisulation, poor head control :

a. Anterior horn cells disorder

Answers

1)C	**2)C**	**3)A**	**4)B**	**5)D**	**6)C**
7)??D	**8)B**	**9)D**	**10)E**	**11)C**	**12)B**
13)B	**14)A**	**15)A**	**16)A**	**17)A**	**18)A**
19)A	**20)D**	**21)?**	**22)**	**23)B**	**24)C**
25)A	**26)E**	**27)C**	**28)B**	**29)D**	**30)C**
31)C	**32)D**	**33)C**	**34)E**	**35)B**	**36)C**
37)C	**38)A**	**39)C**	**40)C**	**41)C**	**42)C**
43)D	**44)B**	**45)A**	**46)B**	**47)B**	**48)A**
49)A	**50)A**	**51)A**	**52)A**	**53)A**	**54)A**
55)A	**56)A**	**57)A**	**58)A**	**59)A**	**60)A**